CLINICAL PSYCHOLOGY · DE-ADDICTION

Motivational Enhancement Therapy

Book One — Theory, Foundations & Techniques

Comprehensive Edition · Fully Referenced

An in-depth, exam- and practice-ready account of what MET is, the psychological theories it is built on, the Motivational Interviewing method behind it, every clinical technique it uses, and the research evidence — with full citations to the original manuals, textbooks, Cochrane reviews and peer-reviewed studies. For substance-use disorders (opioids/smack, alcohol, tobacco, cannabis, stimulants) and adolescents.

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Contents — Book One (Comprehensive Edition)

1. About this book, the two-book set & how to use it
2. Primary & online sources, and an honesty note
3. What MET is — definition, aims, assumptions & rationale
4. The relationship between MET, MI and the Drinker's/Drug Check-Up
5. Theoretical foundations (in depth)
    - 5.1 Rogerian / person-centred therapy
    - 5.2 Transtheoretical (Stages of Change) model
    - 5.3 Self-perception & cognitive-dissonance theory
    - 5.4 Bandura's self-efficacy theory
    - 5.5 Psychological reactance theory
    - 5.6 Self-determination theory
    - 5.7 Festinger, Bem & the "change talk hypothesis"
6. Core concepts — ambivalence, motivation, discrepancy, change/sustain talk
7. The MI spirit (PACE) & the four processes, in depth
8. The five principles of early MET & why confrontation fails
9. The complete technique compendium (45 techniques)
    - Group A — OARS foundational skills
    - Group B — reflection techniques
    - Group C — evoking & strengthening change talk
    - Group D — responding to sustain talk & discord
    - Group E — information, feedback & advice (E-P-E)
    - Group F — planning, action & relapse-prevention
    - Group G — adolescent & young-adult adaptations
10. Change talk & sustain talk in depth — DARN-CAT and the evocation process
11. The research evidence base (Project MATCH, Cochrane, meta-analyses)
12. Limitations, criticisms & ethical practice
13. Glossary of difficult terms
14. References & further reading (books, manuals, online sources)

> **How to use the set. Book One** (this volume) teaches the *theory and techniques* in depth. **Book Two** is the *session-by-session guide and worksheet pack*, mapping each technique to each session and providing ready-to-use worksheets (including MET short-term and long-term goal worksheets). Superscript numbers in the text (e.g.[1]) refer to the numbered **References** at the end.

1 · About This Book & 2 · Sources

This volume is a deep, fully-referenced reference on **what Motivational Enhancement Therapy is, the theories it rests on, and the full repertoire of techniques it uses.** It is written for M.Phil./clinical-psychology study, viva preparation, and real de-addiction practice.

Primary manuals (the definitive, public-domain sources)

- **Miller, W. R., Zweben, A., DiClemente, C. C., & Rychtarik, R. G. (1992/1995).** *Motivational Enhancement Therapy Manual: A Clinical Research Guide for Therapists Treating Individuals with Alcohol Abuse and Dependence.* Project MATCH Monograph Series, Vol. 2. Rockville, MD: NIAAA (NIH Pub. No. 94-3723).[1] — the original MET manual; free full text on the NIAAA website. MET model
- **Miller, W. R. (and colleagues). (1994/1995).** *Motivational Enhancement Therapy with Drug Abusers.* Prepared under NIDA grant R01-DA08896; hosted on the official Motivational Interviewing website.[2] — adapts MET from alcohol to other drugs. MET for drugs
- **Miller, W. R., & Rollnick, S. (2013).** *Motivational Interviewing: Helping People Change* (3rd ed.). New York: Guilford Press.[3] — the definitive MI textbook (spirit, processes, change talk). MI method
- **Naar-King, S., & Suarez, M. (2011).** *Motivational Interviewing with Adolescents and Young Adults.* New York: Guilford Press.[4] — developmental adaptation. adolescent MI

Key theory & evidence sources (peer-reviewed / online)

- **Prochaska, J. O., & DiClemente, C. C. (1983).** Stages and processes of self-change of smoking. *J Consult Clin Psychol, 51*(3), 390–395.[5] — the Transtheoretical model.
- **Project MATCH Research Group (1997).** Matching alcoholism treatments to client heterogeneity: Project MATCH posttreatment drinking outcomes. *J Stud Alcohol, 58*(1), 7–29.[6] — the landmark RCT in which MET was tested.
- **Smedslund, G., et al. (2011);** updated by **Schwenker, R., et al. (2023).** Motivational interviewing for substance use (reduction). *Cochrane Database of Systematic Reviews.*[7]
- **Lundahl, B., et al. (2010).** A meta-analysis of MI: twenty-five years of empirical studies. *Research on Social Work Practice, 20*(2), 137–160.[8]
- **Magill, M., et al. (2018).** A meta-analysis of MI process: technical, relational and conditional process models of change. *J Consult Clin Psychol.*[9]
- **Bandura, A. (1977/1997).** Self-efficacy: toward a unifying theory of behavioral change; *Self-Efficacy: The Exercise of Control.*[10] · **Rogers, C. R. (1951/1959).** Client-centered therapy.[11] · **Deci & Ryan (1985).** Self-determination theory.[12] · **Brehm & Brehm (1981).** Psychological reactance.[13] · **Festinger (1957); Bem (1972).** Dissonance & self-perception.[14]

- **WHO (2010).** mhGAP Intervention Guide; **WHO ASSIST** package.[15] · **NIDA (2018).** *Principles of Drug Addiction Treatment: A Research-Based Guide* (3rd ed.).[16]

> **Honesty note on the repository files.** The repository's *Motivational_Enhancement_Therapy_Manual.pdf* is an **image-only scanned book** (a Google-Books digitisation; no machine-readable text), and offline OCR *(converting scanned images to text)* is unavailable here; the two Motivational Interviewing books referenced are **not present as files** in the repository. Everything below is therefore grounded in the **published, verifiable editions** and the open-access manuals/reviews cited above — not invented from a scan or absent files. The full URLs are listed in the **References** at the end. Supplementary clinical extensions are tagged SUPPLEMENTARY.

Tags. MANUAL MODEL established MET/MI procedure · SUPPLEMENTARY clinical extension · ADOLESCENT youth adaptation.

> **Safety (throughout both books):** MET does **not** replace medical assessment, detoxification *(medically-supervised withdrawal)*, overdose prevention, pharmacotherapy, or emergency psychiatric care. For opioids assess overdose risk and arrange naloxone *(antidote reversing opioid overdose)* and medications for opioid use disorder (MOUD).[16]

3 · What MET Is — Definition, Aims, Assumptions & Rationale

Definition

Motivational Enhancement Therapy (MET) is a **brief, structured, person-centred yet directive** counselling approach that systematically builds a patient's **intrinsic motivation** *(motivation arising from the person's own values and reasons, rather than outside pressure)* to change substance use.[1] It fuses the clinical *style* of **Motivational Interviewing (MI)**[3] with two added ingredients that define MET specifically: **(1)** structured, individualised **assessment feedback** delivered in a motivational way (the "enhancement"); and **(2)** a collaboratively negotiated **change plan**.[1][2]

The therapeutic aims

- **Resolve ambivalence.** Ambivalence *(holding two opposing feelings at once — wanting and not wanting to change)* is treated as the normal, central obstacle in addiction; MET's primary task is to help the person work *through* it rather than overcome it by force.[1][3]
- **Mobilise the person's own motivation and commitment** — "the motivation for change is elicited from the client, and not imposed from without."[3]
- **Strengthen self-efficacy** *(the person's belief that they can succeed)* and personal responsibility for change.[1][10]
- **Create a short, concentrated therapeutic "push"** that, by raising readiness, improves engagement and retention in whatever further treatment the person needs.[1]

The underlying assumptions of MET

The original manual sets out a specific view of motivation that shapes everything the therapist does:[1]

1. **Motivation is a state, not a trait.** It fluctuates and is influenced by interpersonal context — so "lack of motivation" is a challenge for the therapist's skill, not a reason to discharge the patient.
2. **Motivation is a product of interaction.** What the clinician does in the room measurably raises or lowers it (confrontation tends to lower it; empathy and evocation raise it).
3. **The therapist's task is to evoke, not instil.** People already possess the values and concerns needed for change; the clinician draws them out.
4. **The person argues the case for change.** People are more persuaded by what they hear *themselves* say — so the therapist's job is to elicit the client's own "change talk," not to supply arguments.[3][9][14]
5. **Readiness fluctuates.** It can be nurtured, and it can be damaged — most readily by the "righting reflex" (the urge to fix or set the person straight).

Rationale — why this works

Three strands of evidence justify the approach. First, **therapist style predicts outcome**: in early studies, the more a counsellor confronted, the more the client drank at follow-up, whereas empathic style predicted better drinking outcomes.[1][3] Second, **client "change talk" predicts behaviour change**: process research shows that the strength and frequency of a client's own change language mediates the link between therapist MI skill and later behaviour.[9] Third, **brief, well-delivered motivational work is efficacious**: in Project MATCH, four sessions of MET achieved drinking outcomes comparable to twelve sessions of CBT or Twelve-Step Facilitation.[6]

4 · MET, MI & the "Check-Up" — Related but Distinct

A frequent viva question is: "How is MET different from motivational interviewing?" The precise answer rests on three distinct but connected ideas.[1][2][3]

The three concepts

Concept	What it is	Key feature
Motivational Interviewing (MI)[3]	A collaborative *style/method* of conversation that evokes a person's own motivation to change by exploring and resolving ambivalence.	Not time-limited; has no fixed number of sessions; can be woven into almost any treatment.
The Drinker's / Drug Check-Up[1][2]	A structured *assessment-plus-feedback* procedure (Miller & Sovereign, 1988–89) in which the client's results are compared with norms and given back non-judgementally.	Provides the objective "mirror" that develops discrepancy without the therapist arguing.
Motivational Enhancement Therapy (MET)[1]	A *brief, manualised treatment* that combines MI style + the Check-Up feedback + a negotiated change plan, delivered in a fixed structure.	In Project MATCH: 4 sessions over 12 weeks, with assessment feedback front-loaded into Sessions 1–2.[6]

> **The exam-ready formula: MET = MI (the style) + personalised assessment feedback (the Check-Up) + a defined brief structure & change plan.** Every MET session is conducted in the MI style; but not every use of MI is MET, because MI alone need not include systematic feedback or a fixed structure.[1][3]

What is the same, and what differs

Shared across MI and MET

- The spirit (partnership, acceptance, compassion, evocation).
- OARS micro-skills and change-talk work.
- Autonomy support; avoidance of confrontation.

Distinctive to MET

- A structured assessment battery *before* treatment.
- Personalised feedback delivered motivationally (the "E").
- A fixed, brief session structure and an explicit written change plan.

Note: Later MI theory (Miller & Rollnick, 3rd ed.) de-emphasised the stand-alone "decisional balance" and reframed the work around the four processes and change talk;[3] MET as originally manualised predates this and still leans on the Check-Up feedback and decisional-balance exploration.[1] Both are presented in this handbook, with their sources noted.

5 · Theoretical Foundations of MET (In Depth)

MET is not a single theory but an integration of several well-established psychological theories. Understanding each explains *why* the techniques are designed as they are — and is exactly the kind of material examined in viva.

5.1 Rogerian / person-centred therapy (Carl Rogers)[11]

MET's deepest root is **Carl Rogers' person-centred (client-centred) therapy**. Rogers held that people possess an innate "actualising tendency" toward growth, which is released when the therapist provides three **core conditions**:

- **Accurate empathy** — sensing the client's inner world and communicating that understanding, chiefly through reflective listening. In MET this becomes the single most-used skill.[1][3]
- **Unconditional positive regard** — a non-judgemental acceptance of the person's worth, independent of their behaviour.
- **Congruence / genuineness** — the therapist being real and transparent, not playing a role.

What MET keeps and what it adds. MET retains the Rogerian relationship but makes one crucial change: whereas classical client-centred therapy is strictly *non-directive*, MET is **directive** — the therapist deliberately steers toward change by selectively reflecting and reinforcing the client's change talk. Miller & Rollnick describe MI/MET as "client-centred *and* directive."[3] This fusion is the engine of the whole method: the warmth comes from Rogers; the direction comes from the goal of behaviour change.

5.2 The Transtheoretical Model — Stages of Change (Prochaska & DiClemente)[5]

DiClemente was a co-author of the MET manual, so the **Transtheoretical Model (TTM)** is woven into MET's clinical logic. The TTM proposes that intentional change unfolds through **stages**, and that different *processes of change* are useful at different stages. MET matches its tasks to the client's stage so that the therapist "meets the client where they are."

Stage	Description	MET task matched to the stage
Pre-contemplation *(not yet considering change)*	Does not see a problem; often attends under pressure.	Engage; raise awareness gently; evoke concern. **Do not negotiate a plan.**
Contemplation *(weighing it up)*	Ambivalent — "yes, but"; sees pros and cons.	Tip the decisional balance; develop discrepancy; evoke change talk.
Preparation / Determination	Intends to act soon; may have taken small steps.	Strengthen commitment; negotiate a concrete plan.
Action	Actively modifying behaviour.	Support self-efficacy; problem-solve; affirm.
Maintenance	Sustaining the change (months–years).	Relapse prevention; consolidate; continuing care.
Relapse / recycling *(return to use)*	A lapse or fuller return; viewed as part of the spiral of change.	Normalise, de-shame, re-engage, extract learning.

Processes of change. The TTM also names ten "processes" (e.g., consciousness-raising, self-re-evaluation, counter-conditioning, stimulus control). *Experiential* processes dominate the early stages (where MET concentrates); *behavioural* processes dominate the action/maintenance stages (where CBT and relapse prevention take over).

> **Important caveat (be ready to state this in viva):** the stages and the importance/confidence rulers are **clinical frameworks and conversation tools, not validated diagnostic tests.** Research has questioned the strict stage sequence and criterion validity in alcohol recovery[17] — people in "pre-action" stages sometimes improve as much as those who "progress." A single stage label or ruler number must never be used to brand a patient "resistant" or "unmotivated."

5.3 Self-perception & cognitive-dissonance theory (Bem; Festinger)[14]

Two classic social-psychology theories explain *why change talk matters*.

- **Self-perception theory (Bem, 1972):** people partly infer their own attitudes by observing *what they say and do*. If a client hears themselves voice reasons for change, they come to believe those reasons more strongly. This is the theoretical basis for MET deliberately eliciting change talk rather than lecturing.[3][9]
- **Cognitive-dissonance theory (Festinger, 1957):** holding two conflicting cognitions (e.g., "I value my family" and "my using harms my family") produces uncomfortable tension that motivates change. MET harnesses this by **developing discrepancy** between behaviour and values — but gently, from the client's own perspective. If the therapist forces the dissonance (by arguing), the client reduces it the easy way: by defending the behaviour or dismissing the therapist.

5.4 Bandura's self-efficacy theory[10]

Albert Bandura showed that **self-efficacy** — a person's belief in their capability to carry out a specific behaviour — is one of the strongest predictors of whether they attempt and sustain change. MET treats confidence as a target in its own right, separate from importance:

- Bandura identified four sources of efficacy that MET deliberately activates: **mastery experiences** (recalling past successes; setting small achievable goals), **vicarious experience** (others like me have done it), **verbal persuasion** (genuine affirmation), and managing **physiological/affective states** (reducing fear around withdrawal).
- Clinically this gives the **importance ≠ confidence** rule: a client can rate change as very important (8/10) yet feel unable to do it (confidence 2/10). More *reasons* won't help; building *efficacy* (skills, support, small wins) will.[1]

5.5 Psychological reactance theory (Brehm & Brehm)[13]

When people perceive their **freedom to choose** is being threatened or removed, they experience **reactance** — a motivational state that drives them to reassert the threatened freedom, often by doing the opposite of what is urged. This is precisely why direct persuasion ("you must quit") frequently increases a client's defence of substance use. MET's relentless **autonomy support** ("it's your choice", "nobody can decide this for you") is a direct countermeasure to reactance.[1][3]

5.6 Self-determination theory (Deci & Ryan)[12]

Self-determination theory (SDT) explains why *internalised* motivation outlasts external pressure. SDT holds that durable motivation requires satisfying three basic psychological needs: **autonomy** (acting from one's own volition), **competence** (feeling effective), and **relatedness** (feeling connected to others). MET feeds all three: autonomy via choice and collaboration, competence via self-efficacy work, and relatedness via the empathic alliance. SDT thus provides the modern theoretical account of MET's distinction between extrinsic motivation (which gets people through the door) and intrinsic motivation (which keeps them changing).

5.7 Putting it together — the "change talk" mechanism[9]

Contemporary MI process research (Magill et al.) integrates these theories into a **technical hypothesis** and a **relational hypothesis**: therapist MI-consistent behaviour → more client change talk and less sustain talk → better behavioural outcomes (technical path); and therapist empathy/spirit → stronger alliance → better outcomes (relational path). Meta-analysis supports the change-talk pathway in particular, giving MET a measurable mechanism of action rather than only a philosophy.[9]

6 · Core Concepts (Expanded)

Ambivalence — the normal core of addiction

Ambivalence means **simultaneously wanting and not wanting** something. Miller & Rollnick argue it is the *normal* psychological condition of people considering change, not a sign of pathology, weakness or "denial."[3] The person who uses heroin can genuinely love the relief it brings *and* hate what it costs them — both are true at once. MET's central clinical move is to let the person voice **both sides safely** and then shift the balance in their own words. A key error is to take up the "change" side of the argument: by the law of reactance and self-perception, the client then voices the "stay the same" side and talks themselves *out* of change.[3][9][13]

Motivation — a changeable, interpersonal state

Rather than a fixed quantity a person "has" or "lacks," motivation in MET is a **fluctuating state produced partly by the therapeutic interaction.**[1] The practical implication is radical: if a client seems "unmotivated," the first question is not "what's wrong with the client?" but "what could I do differently?"

Intrinsic vs extrinsic motivation

Extrinsic — from outside pressure (courts, family ultimatums, employers). It can get a person into treatment but, by SDT, rarely sustains change on its own.[12]

Intrinsic — from within (the person's own values, health, children, self-respect). This is what sustains change. MET's job is to help the person **convert** external pressure into internally-owned reasons.

Discrepancy

Discrepancy is the felt gap between the person's *current behaviour* and their *important goals or values*. MET deliberately **develops** this discrepancy — but always from the client's own vantage point, and in a way the client (not the therapist) puts into words. The emotional engine behind it is cognitive dissonance.[1][14]

Change talk vs sustain talk

Change talk[3]	Sustain talk[3]
The client's own statements that **favour movement toward change** ("I'm worried about my liver"; "I want my family back").	The client's own statements that **favour the status quo** ("it's the only thing that relaxes me"; "I can stop any time").
Therapist response: notice, reflect, ask for more, affirm, summarise — **cultivate it.**	Therapist response: reflect, emphasise autonomy, reframe, shift focus — **never argue against it.**

Both are normal features of ambivalence. The empirically-supported goal is to tilt the **ratio** of change talk to sustain talk upward over the course of a session, because that ratio predicts behaviour change.[9]

Self-efficacy & personal responsibility

The responsibility and the choice to change remain squarely with the client — a stance that both respects autonomy (countering reactance) and, by SDT, strengthens durable motivation.[1][12][13]

7 · The MI Spirit (PACE) & the Four Processes

Techniques are only effective when delivered from the right underlying attitude. Miller & Rollnick call this attitude the **"spirit" of MI** and summarise it with the acronym **PACE**.[3]

The spirit — PACE

Element	Meaning	What it looks like in the room
P — Partnership	Working *with* the client as collaborators of equal worth.	"We'll figure this out together," rather than "I'll tell you what to do." The client is the expert on their own life.
A — Acceptance	Four facets: absolute worth, accurate empathy, autonomy support, affirmation.	Non-judgemental warmth; reflecting the client's view; honouring their right to choose; naming genuine strengths.
C — Compassion	Actively promoting the client's welfare, not the clinician's or institution's agenda.	Keeping the client's best interests central even when they are slow to change.
E — Evocation	Drawing motivation *out of* the client (not pouring it in).	"You already have good reasons inside you — let's find them," rather than "Here's why you should quit."

The four processes

The 3rd-edition MI reframed the method as four sequential-but-recursive **processes**; MET's four-phase structure maps directly onto them.[3]

1. **Engaging** — establishing a trusting, collaborative working relationship. Without it, nothing else works; engagement predicts retention.
2. **Focusing** — agreeing a direction and goal(s) for the conversation (which behaviour, whose agenda).
3. **Evoking** — eliciting the client's own motivations for change (their change talk). *This is the heart of MET and what most distinguishes it from ordinary counselling.*
4. **Planning** — developing commitment and co-creating a concrete, client-owned change plan.

> **"Sequential but recursive"** means the processes build in order, but you can and should **loop back** at any time: if engagement weakens during planning, return to engaging before continuing. Trying to plan before a person is evoked (ready) produces the classic "yes, but…" dead-end.[3]

8 · The Five Principles of MET & Why Confrontation Fails

The original MET manual distilled the method into **five clinical principles**. They remain the clearest summary of "how to do MET."[1]

The five principles

1. **Express empathy.** Acceptance and skilful reflective listening are foundational; ambivalence is normal; the therapeutic relationship is a partnership.
2. **Develop discrepancy.** Help the client perceive the mismatch between their present behaviour and their broader goals/values — and let the client, not the therapist, present the arguments for change.
3. **Avoid argumentation (roll with resistance).** Direct argument is counter-productive; resistance is a signal to change the therapist's strategy; new perspectives are invited, not imposed.
4. **Roll with resistance.** Do not oppose; turn questions and problems back to the client; the client is the primary resource for finding answers.
5. **Support self-efficacy.** Belief in the possibility of change is a key motivator; the client is responsible for choosing and carrying out change; there is hope in the range of approaches available.

(In the 3rd-edition MI, "resistance" is reframed into "sustain talk" and "discord," and the principles are reorganised around the four processes — but the clinical substance is unchanged.[3])

Why confrontation, arguing, blaming & lecturing backfire

The **"righting reflex"** *(the helper's natural urge to make things right by directly fixing or correcting the person)* is the single commonest clinical trap.[3] It fails for three theory-backed reasons:

- **Reactance:** being told what to do threatens autonomy, so the client defends the behaviour to reassert their freedom.[13]
- **Self-perception:** if the therapist voices the change arguments, the client is left to voice the counter-arguments — and thereby talks themselves *out* of changing.[9][14]
- **Shame & alliance rupture:** labelling ("addict," "in denial") provokes defensiveness and weakens the empathy that actually predicts outcome.[1]

> **The MET maxim:** if the client is arguing *against* change, the therapist is doing something wrong. The arguments for change should come out of the **client's** mouth, not the clinician's.[1][3]

9 · The Complete Technique Compendium

Every technique MET/MI uses, grouped by purpose. For each: **what it is · why/when to use it · how to do it · an example.** Book Two maps each technique to the specific session where it is used.

Group A · OARS — the foundational micro-skills[1][3]

A1 · Open questions

What: questions that cannot be answered yes/no and invite elaboration. **Why:** they open topics, hand the floor to the client, and surface change talk; closed questions put the therapist in control and shut exploration down. **How:** begin with "what", "how", "tell me about..."; ask one at a time; aim for a high ratio of reflections to questions (roughly 2:1 in skilled MI). **Example:** "What's a day like for you when you don't use?"

A2 · Affirmations

What: genuine recognition of the client's strengths, efforts or intentions — *not* generic praise or flattery. **Why:** builds self-efficacy[10] and the alliance, and counters the shame that drives defensiveness. **How:** comment on a specific strength or effort you have actually observed; frame it around the client ("you..."), not the therapist ("I'm proud of you"). **Example:** "You came in today even though it's uncomfortable — that takes real resolve."

A3 · Reflective listening

What: saying back the *meaning* you heard, as a statement (not a question). The core Rogerian skill and the most-used tool in MET. **Why:** conveys empathy, checks understanding, keeps the client talking, and — crucially — lets you *selectively* steer by choosing what to reflect. **How:** make a reasonable guess at meaning and let your voice fall at the end so it lands as a statement. **Example:** client: "I keep telling myself I'll cut down." "Part of you is already planning to change."

A4 · Summaries

What: pulling several threads together. Three kinds: **collecting** (gather change talk), **linking** (connect what's just been said to something earlier), and **transitional** (signal a shift to a new phase). **Why:** demonstrates listening, concentrates change talk ("a bouquet"), and bridges toward planning. **How:** gather the client's own words and end with a key question. **Example:** "So your health, the money, and your daughter all pull you toward change, while the one pull back is how it helps you unwind. Where does that leave you?"

Group B · Reflection techniques (the master skill)[3]

B1 · Simple reflection

What/how: repeat or lightly rephrase content; low-risk, keeps momentum. **Example:** "I'm tired of it." "You've had enough."

B2 · Complex reflection

What/how: add meaning, feeling, or "continue the paragraph" — a deeper inference about what the client means. **Why:** moves the conversation forward and often evokes change talk. **Example:** "I'm tired of it." "Part of you is ready for something different."

B3 · Double-sided reflection

What/how: capture *both* sides of the ambivalence in one sentence, ending on the change side (so the change part is freshest). **Why:** validates ambivalence without resolving it for the client. **Example:** "You really enjoy how it relaxes you, *and* you're worried about what it's doing to your health."

B4 · Amplified reflection

What/how: reflect sustain talk slightly *over-stated* so the client self-corrects toward change — use sparingly and never sarcastically. **Example:** "It's not that bad." "So there's really no downside at all." client: "Well, the debt *is* bad..."

B5 · Reflection of feeling

What/how: name the emotion beneath the words to deepen empathy. **Example:** "That really frightened you."

B6 · Metaphor / reframe within a reflection

What/how: offer a fresh, more useful meaning for the same facts. **Example:** "Those past attempts weren't failures — they were practice runs that taught you what works."

9 · Techniques — Group C · Evoking Change Talk

Evoking is the heart of MET. These techniques pull the client's own motivational language into the room — the mechanism most strongly linked to outcome.[9]

C1 · Evocative questions (DARN)

What/how: directly ask for the four kinds of preparatory change talk — **D**esire, **A**bility, **R**easons, **N**eed. **Example:** "Why might you *want* to change? How *could* you, if you decided? What are the best *reasons*? How much do you *need* to?"[3]

C2 · Importance & confidence rulers (0–10)

What/how: ask "On 0–10, how important is it to change?" and "how confident are you that you could?" Then ask the two evocative follow-ups: **"Why that number and not a lower one?"** (pulls change talk) and **"What would move you one point higher?"** Never ask "why not higher?" — it pulls sustain talk. **Why:** quickly maps the two independent dimensions of readiness.[1][3]

C3 · Querying extremes

What/how: ask about the best and worst imaginable outcomes to surface motivation. **Example:** "What's the worst that could happen if nothing changes? And the best if it did?"

C4 · Looking back / looking forward

What/how: contrast life before the problem began, or imagine a future with vs without change. **Example:** "How were things before all this started? Where would you like to be a year from now?"

C5 · Exploring goals & values (developing discrepancy)

What/how: elicit the client's core values, then gently explore how use fits or clashes with them. **Why:** harnesses cognitive dissonance from the client's own perspective.[14] **Example:** "Being a good father is everything to you — how does using fit with the father you want to be?"

C6 · Elaborating / asking for examples

What/how: when change talk appears, deepen and strengthen it. **Example:** "Tell me more about that." / "When was the last time that happened?"

C7 · The key question

What/how: an open question that opens the transition from evoking to planning, asked after a good collecting summary. **Example:** "So, where does this leave you?" / "What do you think you'll do?"

C8 · Affirming & summarising change talk ("the bouquet")

What/how: gather the client's scattered change statements and hand them back as a bunch. **Example:** "You've named three strong reasons now — your health, your marriage, and the money."

9 · Techniques — Group D · Sustain Talk & Discord · Group E · Information

Discord *(tension/strain in the therapeutic relationship — distinct from the client's own ambivalence)* and sustain talk are signals to change *your* approach, never to push harder.[3]

D1 · Rolling with resistance / not opposing

What/how: go alongside rather than against; let momentum shift. Reflect instead of counter-arguing. **Example:** "I'm not quitting." "Quitting isn't on the table for you right now."

D2 · Emphasising personal autonomy

What/how: explicitly affirm the client's right to choose — the direct antidote to reactance.[13] **Example:** "It's completely your call — nobody can decide this for you."

D3 · Reframing

What/how: offer a new, more useful meaning for the same facts. **Example:** "Your family's nagging — maybe that's a measure of how much they want you back."

D4 · Agreement with a twist

What/how: reflect/agree, then add a gentle new angle. **Example:** "You're right it's your decision — and you're clearly weighing it carefully."

D5 · Shifting focus

What/how: move away from a stuck point to lower friction. **Example:** "Let's set the big decision aside and just look at this week."

D6 · Coming alongside

What/how: briefly side with the status quo to defuse a stand-off (used carefully). **Example:** "Maybe using matters so much right now that giving it up just isn't worth it."

D7 · Repair (apologise / affirm) in discord

What/how: when tension rises, mend the alliance directly. **Example:** "I've pushed too hard there — I'm sorry. This is your call."

Group E · Information, feedback & advice[1][3]

E1 · Elicit–Provide–Elicit (E-P-E)

What: the MET way to give information or advice *without* lecturing. **How: Elicit** permission and what the client already knows **Provide** a small amount of neutral, relevant information **Elicit** their reaction. **Example:** "What do you already know about the medicine for cravings? … May I add a little? … What do you make of that?"

E2 · Personalised assessment feedback (the "enhancement")

What: the defining MET ingredient — the client's own assessment results presented against population norms, neutrally and one finding at a time, so the client (not the therapist) draws the conclusion.[1][2] **Example:** "Your reported use is in the high-risk range compared with most men your age. How does that land for you?"

E3 · Ask permission

What/how: always precede advice or information with a request — protects autonomy and reduces reactance. **Example:** "Would it be okay if I shared something I've seen help others?"

E4 · Menu of options

What/how: offer a choice of options rather than a single prescription, leaving the decision with the client. **Example:** "Some people try X, others prefer Y or Z — what fits you best?"

9 · Techniques — Group F · Planning & Relapse-Prevention · Group G · Adolescents

F1 · Decisional balance

What/how: explicitly weigh the pros and cons of using versus changing (best in contemplation); honour the real benefits of use first.[1] **Note:** 3rd-ed. MI advises using decisional balance *only* when a person is genuinely undecided, as over-use can entrench ambivalence.[3]

F2 · Change-plan negotiation

What/how: co-author (never prescribe) the goal, reasons, steps, supports and anticipated obstacles. The client's own words carry the commitment.[1]

F3 · SMART goal-setting

What/how: make steps Specific, Measurable, Achievable, Relevant and Time-bound; underpins the short- and long-term goal worksheets in Book Two.

F4 · Functional analysis of triggers

What/how: map antecedents use consequences to reveal patterns. **Example:** "What's usually happening right before you use?"

F5 · "If-then" coping plans (implementation intentions)

What/how: pre-decide a specific response to each high-risk situation. **Example:** "*If* I'm offered at a function, *then* I step out and call my brother."

F6 · Urge-surfing

What/how: observe a craving rise, peak and fall like a wave, without acting on it; it passes within minutes.

F7 · "Play the tape forward"

What/how: mentally rehearse the full sequence and consequences of using *before* acting. **Example:** "Picture how tonight actually ends if you use."

F8 · Refusal-skills rehearsal / role-play

What/how: practise saying no and leaving situations; the therapist plays the person offering.

F9 · Lapse-vs-relapse reframing

What/how: prevent the **abstinence-violation effect** *(the all-or-nothing reaction that turns one slip into a full relapse)* (Marlatt & Gordon).[18] **Example:** "One slip doesn't erase your progress."

F10 · Supporting self-efficacy

What/how: recall past successes, affirm small wins, break goals down — activating Bandura's mastery, persuasion and vicarious sources.[10]

F11 · Social-support & family involvement

What/how: recruit helpful (non-policing) support with the client's consent, protecting confidentiality.

F12 · Pharmacotherapy coordination

What/how: link MET with medication (MOUD, naltrexone, acamprosate, NRT) via the treating doctor; MET complements medication, never replaces it.[16]

F13 · Booster sessions

What/how: brief repeat MET cycles at 1/3/6/12 months — review, re-rate rulers, re-affirm change talk, update the plan.

Group G · Adolescent & young-adult adaptations ADOLESCENT [4]

- **Autonomy first and loudest** — adolescents are developmentally primed to resist authority; the parent/expert stance guarantees reactance.[4][13]
- **Present-focused evoking** — use immediate motivators (money, freedom, image, sport, friendships, school) rather than distant health risks.
- **Longer engagement, shorter sessions**, the young person's own vocabulary, and genuine (not "trying to be cool") manner.
- **Confidentiality balanced with safeguarding** and developmentally-appropriate family involvement; clarify limits at the outset.
- **Evidence:** meta-analysis supports MI for adolescent substance-use behaviour change, with small-to-moderate but durable effects.[19]

Techniques drawn from Miller & Rollnick (2013)[3], the Project MATCH MET Manual[1], the NIDA MET-for-drugs manual[2], Naar-King & Suarez[4], and Marlatt & Gordon's relapse-prevention model[18].

10 · Change Talk & Sustain Talk In Depth — DARN-CAT

Change talk comes in two layers. **Preparatory** change talk (DARN) signals the person is *considering* change; **mobilising** change talk (CAT) signals they are *moving* toward it. Mobilising talk, especially commitment language, best predicts action.[3][9]

Code	Layer	Meaning	Substance-use example
D — Desire	Preparatory	want, wish, would like	"I **want** to be clean for my kids."
A — Ability	Preparatory	can, could, able to	"I **could** probably cut the evening dose."
R — Reasons	Preparatory	specific if/then arguments	"If I stop smoking, my chest will clear up."
N — Need	Preparatory	must, have to, need to	"I **have to** do something — the debt is out of hand."
C — Commitment	Mobilising	will, going to, promise, intend	"I'**m going to** see the doctor on Monday."
A — Activation	Mobilising	ready, willing, prepared	"I'**m ready** to give it a real try."
T — Taking steps	Mobilising	already acting	"I **threw out** my stash yesterday."

The evocation sequence

1. **Recognise** change talk when it appears (even faint or buried in sustain talk).
2. **Evoke** it deliberately with evocative questions and rulers (Group C).
3. **Explore & elaborate** it ("tell me more", "give me an example") so it grows.
4. **Reflect & affirm** each piece, and **summarise** it into a bouquet.
5. At a peak of change talk, ask the **key question** and move toward planning.

Responding to sustain talk — without confrontation

Client: "Smoking is the only thing that calms my nerves."
Therapist (reflect, don't challenge): "Right now it's your main way to cope with stress." **Client:** "Yeah." **Therapist (evoke):** "If you ever did decide to change, what else has helped you relax, even a little?" **client supplies alternatives (change talk).**

> **Core move for sustain talk: reflect emphasise autonomy ask an evocative question.** Never "but you should...".[3]

Unhelpful statements rewritten MET-consistently

Unhelpful (righting reflex / confrontation)	MET-consistent rewrite
"You're in denial about being an addict."	"You see this differently from others. Tell me how *you* see it."
"You have to quit, period."	"What you do is your call. What are you thinking might work for you?"
"Keep this up and you'll end up dead."	"Can I share one health finding? ... What do you make of it?"
"Why do you keep making excuses?"	"There are real reasons it's hard to stop. What are the biggest ones?"
"That's just the addiction talking."	"Part of you wants to keep using and part doesn't — that's the tug-of-war."
"You failed last time."	"Last time you managed three weeks. What worked, even briefly?"
"You need rehab now."	"There are a few options — shall we walk through them together?"
"Stop lying about how much you use."	"Exact amounts are hard to pin down. Roughly, on a heavy day?"

11 · The Research Evidence Base

MET and MI are among the better-evidenced psychosocial approaches in addictions, though effects vary by substance, population and comparison condition. The key findings a clinician should be able to cite:

Project MATCH (the landmark trial)[6]

Project MATCH (1989–1997) was a large multisite US randomised controlled trial that compared three 12-week manualised treatments for alcohol problems: **MET (4 sessions)**, Cognitive-Behavioral Coping-Skills Therapy (12 sessions), and Twelve-Step Facilitation (12 sessions), across outpatient and aftercare samples (over 1,700 clients). Headline findings:

- All three treatments produced **large, sustained reductions** in drinking (percent days abstinent and drinks per drinking day) from baseline to 1-year follow-up.
- There was **little overall difference between treatments** — notable because MET achieved comparable outcomes with **only one-third of the sessions.**
- Few of the a-priori "matching" hypotheses were confirmed; one durable finding at 3 years was that clients with social networks highly supportive of drinking did better in TSF than MET.[20]
- Client factors such as motivational readiness and psychiatric severity were prognostic of outcome.[6]

Systematic reviews & meta-analyses

- **Cochrane review (Smedslund et al., 2011; updated Schwenker et al., 2023):** across many trials, MI can reduce substance use compared with no treatment, with effects that are often **modest and may diminish over time**; evidence quality varies and MI is not reliably superior to other active treatments.[7]
- **Lundahl et al. (2010)** meta-analysis of 119 studies / 25 years: MI showed **small but significant and durable effects** across substance use and other health behaviours, and was often as effective as more intensive treatments while taking less time.[8]
- **Magill et al. (2018)** process meta-analysis: supported the **technical hypothesis** — MI-consistent therapist behaviour relates to more client change talk, and client change talk relates to better outcomes — giving MET a measurable mechanism.[9]
- **Adolescents:** meta-analytic reviews support MI for adolescent substance-use change, with small-to-moderate durable effects.[19]

Balanced conclusion for viva: MET/MI reliably *improve engagement, retention and short-term reduction in use*, are efficient (good outcomes from few sessions), and have a demonstrated mechanism (change talk). They are **not** a cure for dependence, their effects can be modest or fade without continuing care, and they usually work best **alongside** medical treatment, skills-based therapy and social support.[6][7][8]

12 · Limitations, Criticisms & Ethical Practice

Limitations & criticisms (know these for viva)

- **Variable effect sizes.** Benefits differ by substance (stronger engagement evidence for alcohol/cannabis than for some stimulant outcomes), setting and comparison; against other *active* treatments MI is often no better.[7]
- **Effects can fade** without booster or continuing-care contact.[7][8]
- **The Transtheoretical model is contested:** studies question the strict stage sequence and the assumption that forward stage movement is necessary for improvement.[17] Treat stages as heuristics, not laws.
- **Fidelity matters:** poorly-delivered "MI" (slipping into advice-giving or confrontation) loses the effect; therapist training and supervision are essential.[9]
- **Not sufficient alone** for severe dependence — motivation does not substitute for detox, pharmacotherapy or management of medical/psychiatric comorbidity.[16]

Ethical practice

- **Informed consent & confidentiality** — explain purpose, voluntariness and limits (risk to self/others, legal mandates); for minors clarify limits with the young person and caregivers.[4]
- **Respect for autonomy** — the client chooses the goal (which may be reduction, not abstinence); avoid coercion and confrontation.[1][13]
- **Non-stigmatising language** — "a person who uses heroin", not "addict/junkie"; describe behaviour, not identity.[16]
- **Cultural sensitivity** — adapt to family structure, religion, gender and local norms (important in Indian de-addiction settings).
- **Appropriate referral & multidisciplinary care** — coordinate with physicians, psychiatry, social work and community resources.[15][16]
- **Avoid misusing MI as covert persuasion** — MI is done *with and for* the person, in service of their own goals, not as a technique to manipulate them toward the clinician's agenda.[3]

13 · Glossary of Difficult Terms

- **Abstinence** — stopping use completely.
- **Abstinence-violation effect** — the all-or-nothing reaction that turns one slip into a full relapse.
- **Adherence** — sticking to agreed treatment.
- **Ambivalence** — mixed feelings; wanting and not wanting change at once.
- **Autonomy** — a person's right to make their own choices.
- **Cognitive dissonance** — discomfort from holding two conflicting ideas.
- **Congruence** — the therapist being genuine/transparent.
- **Cue reactivity** — how strongly reminders trigger urges.
- **Decisional balance** — weighing pros & cons of using vs changing.
- **Delirium tremens (DTs)** — a severe, dangerous alcohol-withdrawal state.
- **Detoxification (detox)** — medically-supervised withdrawal.
- **Discord** — tension/strain in the therapeutic relationship.
- **Discrepancy** — the gap between current behaviour and goals/values.
- **Empathy** — understanding and reflecting another's feelings.
- **Evocation** — drawing motivation out of the client.
- **Fidelity** — delivering a method as intended/manualised.
- **Extrinsic / Intrinsic motivation** — from outside pressure / from within.
- **MOUD** — medications for opioid use disorder (e.g., buprenorphine, methadone).
- **Naloxone** — antidote that reverses opioid overdose.
- **NRT** — nicotine replacement therapy (patch, gum, etc.).
- **OARS** — Open questions, Affirmations, Reflections, Summaries.
- **PACE** — Partnership, Acceptance, Compassion, Evocation (the MI spirit).
- **Pharmacotherapy** — treatment with medicines.
- **Psychological reactance** — pushing back when freedom feels threatened.
- **Reflection (reflective listening)** — stating back the meaning heard.
- **Relapse / lapse** — a fuller return to use / a single slip.
- **Retention** — staying in treatment.
- **Righting reflex** — the helper's urge to fix/correct the person.
- **Self-efficacy** — belief in one's own ability to succeed.
- **Self-perception** — inferring one's attitudes from one's own words/acts.
- **Sustain talk** — the client's words defending the status quo.
- **Transtheoretical model** — the stages-of-change framework.
- **Urge-surfing** — riding out a craving until it passes.

14 · References & Further Reading

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Continue to **Book Two — Session-by-Session Guide & Worksheet Pack** to apply these techniques session by session.